

Team Aurora

Documentation does not tell the whole story

“Take the questions with you. This material brings together the concepts and the case so you can continue the discussion after the game.” — Iris

The problem we investigated

In the oncology ICU, documentation suggests uneven adherence across shifts. Management wants more training; staff report duplicate charting. We do not yet know how much of the gap concerns documentation and how much concerns delivery of care.

Mission objectives

- Distinguish documentation gaps from delivery gaps
- Prioritize determinants related to workflow and shifts
- Plan a pilot and monitor reach and fidelity

Initial fictional case data

Measure	Data	What it represents
Documentation · day shift	52/60	Eligible opportunities with a documented risk assessment
Documentation · night shift	18/40	The same criterion, applied to eligible night-shift opportunities
Direct observation	16/30	Observed opportunities in which all core components were delivered
Training	28/32	Eligible staff who attended; attendance does not establish delivery

Educational simulation. Fictional data, patients and characters. Framework applications are educational interpretations, rather than clinical recommendations or a validated instrument.

From knowledge to change

Knowledge-to-Action

KTA connects knowledge creation with an action cycle. Identifying the gap, adapting to context, investigating barriers, selecting strategies, monitoring, evaluating and sustaining are activities that can be revisited.

In your case: Distinguishing documentation and delivery gaps guides the first test.

Avoid: Treating the cycle as a rigid checklist or assuming that publishing the protocol completes implementation.

Source: Graham et al., 2006

Updated CFIR

CFIR organizes determinants into five domains. It supports investigation of barriers and facilitators; it does not automatically select a strategy or replace outcome evaluation.

Domains: innovation; outer setting; inner setting; individuals; process.

In your case: Duplication, access and skills may coexist; compare accounts with observation.

Avoid: Confusing a barrier with the strategy to address it, or classifying an account without defining the project's context.

Source: Damschroder et al., 2022; Reardon et al., 2025

PARIHS and i-PARIHS

PARIHS emphasizes evidence, context and facilitation. i-PARIHS connects innovation, recipients and context through active facilitation: assessing needs, aligning conditions and supporting change.

Facilitation requires action: assessing needs, negotiating conditions, preparing people and monitoring implementation. Appointing someone is not the same as facilitating.

In your case: Negotiating a workflow across shifts is an action; appointing the facilitator is not enough.

Avoid: Appointing a facilitator and assuming that the title, without action and support, resolves implementation.

Source: Harvey and Kitson, 2016

Planning and evaluating with RE-AIM

RE-AIM supports planning and evaluation at individual and organizational levels. Define the unit of analysis, numerator, denominator, source and time point; also investigate representativeness.

Dimension	Study question
Reach	Participation and characteristics of intended recipients.
Effectiveness	Effects of the intervention, including unintended effects.
Adoption	Participation of settings and staff who initiate the practice.
Implementation	Delivery, adaptations and costs.
Maintenance	Continuity at individual and organizational levels.

Application to the mission

Simulated reach: $18/24 = 75\%$; day shift $14/16 = 87.5\%$; night shift $4/8 = 50\%$. The total hides differences. Investigate patient characteristics and opportunities before attributing the difference to the shift.

Indicator worksheet

Record: dimension or outcome; unit of analysis; numerator; denominator; source; time point; responsible person; criterion for reviewing the plan. Analyze who was left out and unintended effects.

Using an overall rate to hide groups not reached, or confusing initial adoption with continuity.

Source: Glasgow et al., 2019. Definitions cover individual and organizational levels; measures depend on the project.

Implementation outcomes

Proctor's taxonomy distinguishes implementation outcomes from service and clinical outcomes. Each measure must specify what is assessed, by whom, at which level and time point.

Outcome	What to investigate
Acceptability	Perception that the practice is satisfactory.
Adoption	Intention, initial decision or initiation of use.
Appropriateness	Perceived relevance and compatibility.
Feasibility	Possibility of delivering the practice in the context.
Fidelity	Delivery as intended.
Cost	Financial impact of implementation.
Penetration	Integration within services and subsystems.
Sustainability	Maintenance or institutionalization.

Your team's study priority

Fidelity: observe eligible opportunities and delivery of core components. Compare observation with documentation. Define completeness criteria, the auditor and the period; a completed screen does not establish care delivery.

Distinguish the measures

Clinical outcome: change in health status, such as pressure injury occurrence. Service outcome: quality or functioning of care. Implementation outcome: embedding the practice. These may be related, but one does not establish the others or, by itself, causality.

Source: Proctor et al., 2011 and 2023.

Revisit the discussion

The context changed

IT will only be able to change the charting system in six weeks. The night shift requests an interim solution without a third record.

Questions to review your reasoning

- Did the team distinguish documentation from delivery?
- Did the strategy include both shifts and preserve core components?
- Does the indicator investigate fidelity without inferring clinical causality?
- Which data supported the choice? Which new evidence would make you revise the plan?
- Did you distinguish determinant, strategy and outcome? Describe an example from the mission.
- How did you preserve core components and adapt workflow to context?

Notes after the discussion

Record the selected strategy, a limitation raised in discussion, a complete indicator and the next step. Also revisit facilitator feedback in the game journal.

This guide presents the case and concepts; it does not automatically reproduce decisions or feedback entered in the game.

My next step: _____

Evidence I still need: _____

Indicator and review criterion: _____

Readings and source locators

Graham et al., 2006. Lost in knowledge translation: time for a map?. *Journal of Continuing Education in the Health Professions*, 26(1), 13–24.

Locator: KTA model and description of the action cycle

<https://doi.org/10.1002/chp.47>

Damschroder et al., 2022. The updated Consolidated Framework for Implementation Research based on user feedback. *Implementation Science*, 17, 75.

Locator: Table 3: updated domains and constructs

<https://doi.org/10.1186/s13012-022-01245-0>

Reardon et al., 2025. The CFIR User Guide: a five-step guide for conducting implementation research using the framework. *Implementation Science*.

Locator: Steps 1–3: define domains, collect and analyze data

<https://doi.org/10.1186/s13012-025-01450-7>

Harvey and Kitson, 2016. PARIHS revisited: from heuristic to integrated framework for the successful implementation of knowledge into practice. *Implementation Science*, 11, 33.

Locator: Table 2, Table 3 and The facilitation construct section

<https://doi.org/10.1186/s13012-016-0398-2>

Glasgow et al., 2019. RE-AIM Planning and Evaluation Framework: Adapting to New Science and Practice With a 20-Year Review. *Frontiers in Public Health*, 7, 64.

Locator: The RE-AIM dimensions: definitions, evolution, and examples section

<https://doi.org/10.3389/fpubh.2019.00064>

Proctor et al., 2011. Outcomes for Implementation Research: Conceptual Distinctions, Measurement Challenges, and Research Agenda. *Administration and Policy in Mental Health*, 38, 65–76.

Locator: Table 1 and Taxonomy of Implementation Outcomes section

<https://doi.org/10.1007/s10488-010-0319-7>

Proctor et al., 2023. Ten years of implementation outcomes research: a scoping review. *Implementation Science*, 18, 31.

Locator: Conceptualization and discussion of relationships between strategies and outcomes

<https://doi.org/10.1186/s13012-023-01286-z>

Material prepared with assistance from artificial intelligence. Reading the sources and academic analysis remain with the professor and students.